



# The intersectionality of sex and ethnicity on primary care management of symptoms of suspected colorectal cancer

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What is the problem being addressed?



1. Colorectal cancer (CRC), the second leading cause of all UK cancer-related deaths, has worse outcomes when diagnostic delays are over two weeks.
2. Ethnic minorities face delays of up to three weeks longer than white counterparts for diagnosis.
3. Females have longer referral times and worse 1 year survival than males, despite ovarian hormones being protective.

Urgent referrals from primary care for diagnosis depend on patient symptoms or faecal immunochemical test (FIT) results.

Therefore, identifying variations in primary care management could inform targeted interventions.

## Aim:

To explore the inequalities associated with the sex-ethnicity intersectionality on primary care management and outcomes in people with suspected CRC.

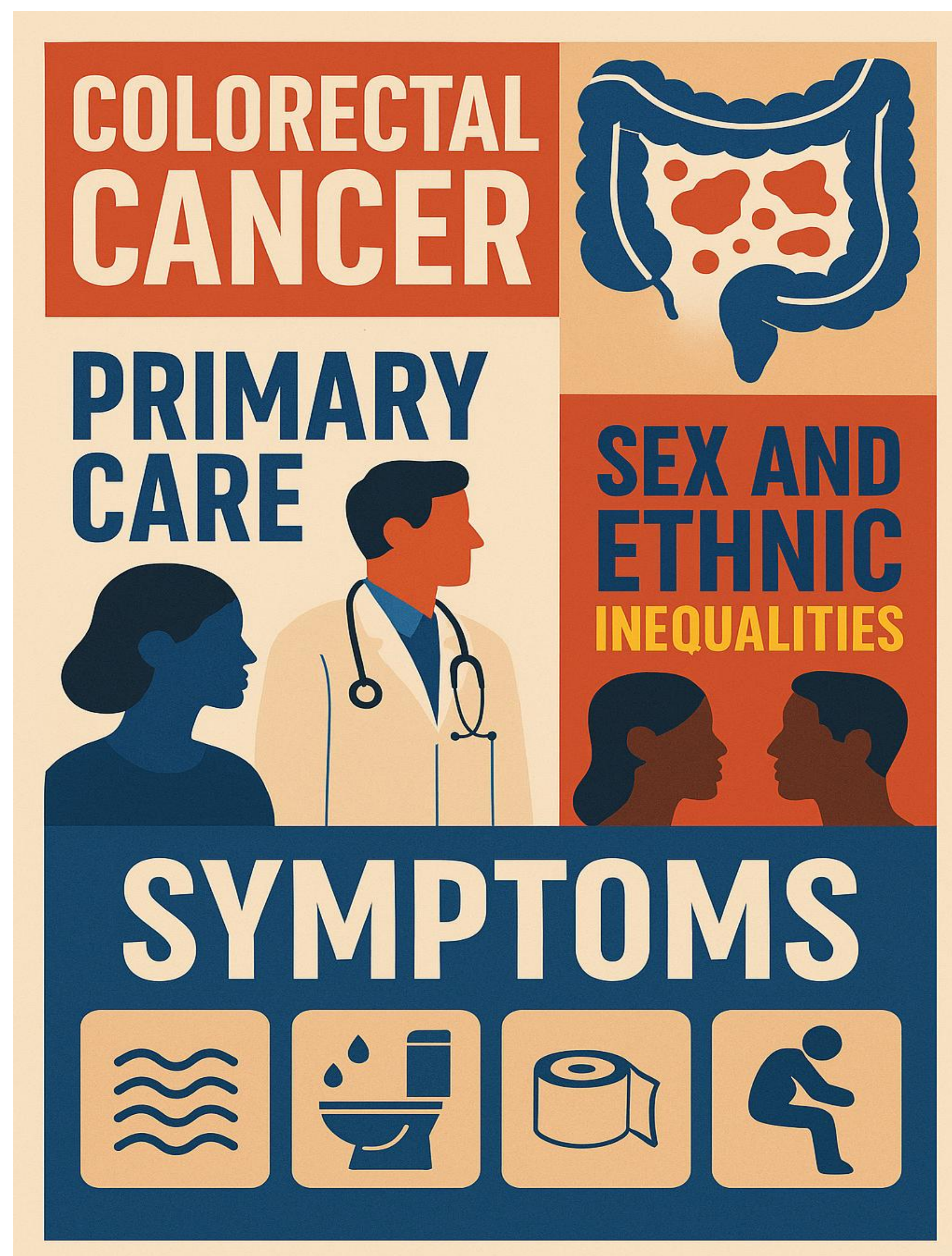
### Work package 1: Systematic review and meta-analysis

I will identify patient, clinician and system factors affecting timeliness of FIT and referrals in primary care patients with suspected CRC symptoms across sex and ethnicities.

### Work package 2: Data study

I will analyse Clinical Research Practice Datalink-Aurum, Hospital Episode Statistics and Office for National Statistics data using regression analysis to investigate sex and ethnic differences in the number and types of tests, treatments and alternate diagnoses for suspected CRC symptoms.

What impact will this have?



How will this be explored?



### Work package 3: Data study

I will analyse the above data using time to event models to investigate if sex and ethnicity are associated with (a) time to FIT (b) time to referral and (c) death in patients with suspected CRC symptoms.

### Work package 4: Focus group and Delphi consensus

I will collate findings through 2 focus group discussions with stakeholders including CRC patients, GPs, colorectal surgeons and cancer charity representatives. Themes will be co-identified with my public and patient involvement group and discussions will explore further research, areas of care inequality and potential interventions. Findings will inform a consensus panel to guide policy and research recommendations.

This will provide critical evidence on CRC detection across sex and ethnicities, informing intervention development and influencing policy. Findings will be reported to the NIHR Policy Research Unit on cancer awareness, screening and early diagnosis and could inform future national guidelines

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